Draft — pending clinical review

Fasting Ramadan safely with diabetes

Plan with your doctor, check your sugar, and know when to break your fast

The 5 things to remember

- See your doctor **6–8 weeks before Ramadan** to plan your fast and your medicines.
- Checking your sugar **does not break the fast**.
- Break your fast at once if sugar is **below 70** or **above 300 mg/dL**, or you feel unwell.
- Do not skip **suhoor**. Eat it as late as you can.
- Never change or stop your medicines on your own.

Can I fast?

Many people with diabetes fast safely. Some people face a high risk of harm. Your doctor will work out a risk score from your sugar control, your medicines, and your health.

Fasting may **not** be safe if you are pregnant and have diabetes, have had severe or frequent low sugar, have had very high sugar or diabetic ketoacidosis, or have serious kidney or heart problems.

Islam allows people not to fast when fasting may harm them. You can discuss this with your doctor and with a religious scholar you trust.

How do I check my sugar while fasting?

- A finger-prick test or a glucose sensor **does not break the fast**.
- Check more often than usual — your doctor will tell you how often.
- Good times to check: before suhoor, mid-morning, midday, mid-afternoon, before iftar, 2 hours after iftar.
- Check **any time** you feel shaky, sweaty, confused, or unwell.

Your sugar reading	What to do
Below 70 mg/dL (3.9 mmol/L)	**Break your fast now.** Treat low sugar (see below).
Above 300 mg/dL (16.7 mmol/L)	**Break your fast now.** Drink water and contact your doctor.
Between these values	Continue your fast if you feel well.

How do I treat low sugar?

- Take **15 grams of fast sugar**: 3 teaspoons of sugar in water, or half a cup (120 ml) of regular juice.
- Check again after 15 minutes. If still below 70, repeat.
- When it is above 70, eat a small meal or snack.

What should I eat?

Suhoor	Iftar
Eat as late as you can, just before Fajr.	Break your fast with 1–3 dates and water.
Choose slow foods: brown bread, oats, ful or beans.	Then a balanced plate: half vegetables, some protein, a small amount of rice or bread.
Add protein: eggs, labneh, cheese or yoghurt.	Keep fried food (sambosa) and sweets (kunafa, qatayef) small and rare.
Drink plenty of water.	Avoid sugary drinks such as Vimto and sweet juices.

- Drink water often between iftar and suhoor.
- Taraweeh prayers count as activity. Avoid hard exercise while fasting, especially near iftar.

What about my medicines?

- Some medicines need a **new dose or time** in Ramadan — especially insulin and tablets such as gliclazide or glimepiride.
- Some tablets, such as dapagliflozin or empagliflozin, need extra care with fluids.
- If you have type 1 diabetes, **never stop insulin**.
- After Ramadan, go back to your usual doses as your doctor advised. Take care with Eid sweets.

Get help now — call 997 or go to the emergency department if:

- Your sugar stays below 70 after two treatments, or you become confused or very drowsy.
- You vomit and cannot keep fluids down.
- High sugar comes with tummy pain, vomiting, or fast deep breathing.
- You have signs of dehydration: very dry mouth, dizziness when standing, very little urine.

My Ramadan plan

My risk level (from my doctor): _____

Medicines at suhoor: _____

Medicines at iftar: _____

My clinic phone number: _____

AA

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This leaflet gives general information. It does not replace advice from your own doctor, who knows your health. Bring it to your next visit and ask about anything that is unclear. Written with AI assistance, checked by the medical editor, and still under review with colleagues.

Last updated: October 2026 · **Next review:** October 2027, or sooner if guidance changes

Sources:

1. IDF-DAR Practical Guidelines: Diabetes and Ramadan. International Diabetes Federation & DAR International Alliance, 2021
2. Afandi B, et al. The 2026 update of the IDF-DAR risk calculator for fasting in people with diabetes. J Diabetes Endocr Pract 2026;9(1)
3. American Diabetes Association. Standards of Care in Diabetes — 2026, Section 5 (Rec 5.32 and Table 5.3, pre-fasting risk assessment) and Section 6 (hypoglycemia). Diabetes Care 2026;49(Suppl 1)
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